

THE NUTRITION CLUB CLINIC

Weight Loss Guide

Evidence-based, sustainable nutrition guidance for losing weight and keeping it off.

The best plan is the one you can keep — tailored to your life and health.

What's inside

Most people already know roughly what to eat. The hard part is doing it consistently for long enough, and not regaining it afterwards. This guide covers the energy balance basics, how to build a plan you can live with, the behaviour side, and what to do when the scale stops moving.

1. How weight loss actually works
2. Setting your targets
3. The four habits that do most of the work
4. Behaviour, environment and mindset
5. Plateaus, exercise and keeping it off

How to use this guide

Set your numbers in section 2, then apply the four habits in section 3 before worrying about anything clever. Give any change at least three weeks before judging it, and weigh yourself the same way each time so the data is comparable.

1. How weight loss actually works

Fat loss requires an energy deficit — taking in less energy than you use, over time. Every successful diet, whatever it is called, works through this one mechanism. What differs is how easy each approach makes the deficit to sustain.

Where your energy goes

Component	Share of daily burn	Can you influence it?
Basal metabolic rate	~60–70%	Somewhat — more muscle raises it modestly
Non-exercise activity (NEAT)	~15–30%	Yes, substantially — steps, standing, fidgeting, chores
Exercise	~5–10%	Yes, but it's smaller than most people assume
Digesting food	~10%	Slightly — protein has the highest thermic effect

Realistic expectations

- A rate of **0.5–1% of body weight per week** preserves muscle and is sustainable.
- Weight is not linear. Water, salt, carbohydrate, hormones and bowel contents can swing the scale 1–2 kg in a day. Judge by weekly averages.
- Faster loss means more muscle lost, more hunger, and higher regain risk.
- Fat comes off the whole body — you cannot spot-reduce a belly or thighs.

Why it gets harder

As you lose weight your body needs fewer calories and adapts by lowering NEAT and increasing hunger hormones. This is normal biology, not failure — it just means the plan needs periodic recalculation.

2. Setting your targets

Step 1 — estimate your maintenance calories

A reasonable estimate: body weight in kg × 30–33 for a mostly sedentary adult, × 33–37 if moderately active. An 85 kg fairly sedentary person is around 2,550 kcal.

Step 2 — set a moderate deficit

Subtract **15–25%**, usually 400–600 kcal. Our 85 kg example would target roughly 2,000 kcal for about 0.5–0.7 kg loss per week. Very low calorie targets produce fast early results and high dropout.

Step 3 — protect muscle with protein

Nutrient	Target	Why it matters in a deficit
Protein	1.6–2.2 g per kg of target body weight	Preserves muscle, the most filling nutrient, highest thermic effect
Fibre	25–35 g per day	Volume and fullness for very few calories; supports gut health
Fat	At least 0.6 g per kg	Hormones, vitamin absorption, satiety and palatability
Carbohydrate	Whatever remains	Fuel for training and daily energy — quality over elimination
Water	2–3 litres daily	Often mistaken for hunger; supports training and digestion

Diet style is personal preference

Low carb, Mediterranean, intermittent fasting, plate method, calorie counting — head-to-head trials find similar average results. The best one is the one that matches your schedule, culture, budget and tastes, because adherence is the variable that actually differs.

3. The four habits that do most of the work

- 1 Protein and vegetables at every meal.** Fill half the plate with non-starchy vegetables and a quarter with protein. This alone controls calories without counting them.
- 2 Drink no calories.** Sugary drinks, juice, sweetened coffees and alcohol add hundreds of calories with no fullness. This is usually the fastest single win.
- 3 Cook most of your meals.** Restaurant and takeaway portions carry far more oil, sugar and salt than you'd use. Cook once, eat twice.
- 4 Walk 7,000–10,000 steps a day.** NEAT is the most underrated lever in weight loss and the first thing the body quietly reduces when you diet.

Practical portion guides using your hand

Food	Portion
Protein	A palm-sized portion per meal (two for larger or very active people)
Vegetables	Two fist-sized portions per meal — more is fine
Carbohydrate	One cupped-hand portion per meal
Fats	One thumb-sized portion per meal (oil, butter, nut butter, cheese)

Filling foods per calorie

Potatoes, eggs, oats, fish, lean meat, Greek yoghurt, pulses, fruit, soups and big salads deliver the most fullness for the fewest calories. Build meals from these and hunger becomes manageable.

You don't need to be perfect

Aim for about 80% of meals aligned with your plan. Room for a restaurant dinner or a slice of cake is what makes a plan survive a year instead of three weeks.

4. Behaviour, environment and mindset

Design your environment

- Keep trigger foods out of the house, or at least out of sight, rather than relying on willpower.
- Prepare food in advance — decisions made while hungry are rarely good ones.
- Use smaller plates and serve from the kitchen rather than the table.
- Eat without screens; distracted eating consistently increases intake.

Sleep and stress

Sleeping under six hours raises ghrelin and lowers leptin, increasing appetite for calorie-dense food, and reduces the proportion of weight lost as fat. Chronic stress drives both cortisol and emotional eating. Sleep and stress management are weight-loss interventions, not side notes.

Emotional eating

Before eating outside a planned meal, pause and name what you feel: bored, tired, anxious, lonely, or genuinely hungry? If it isn't hunger, food won't fix it. Have two or three non-food responses ready — a walk, a call, a shower, ten minutes of something absorbing.

Tracking that helps

- Weigh daily if it doesn't stress you, and compare 7-day averages; weekly if daily numbers derail you.
- Measure your waist monthly — it often moves when the scale doesn't.
- Take photos every four weeks in the same light and clothing.
- Track behaviours too: steps, sessions trained, meals cooked, hours slept.

Change the self-talk

One off-plan meal is a meal, not a failed week. The people who succeed long term are simply the ones who resume the next meal instead of abandoning the month.

5. Plateaus, exercise and keeping it off

When the scale stops for 3+ weeks

- 1 Check your intake honestly for a week — cooking oils, drinks, tastes while cooking and weekends are the usual culprits.
- 2 Recalculate your needs: you are lighter now, so maintenance has fallen.
- 3 Increase daily steps before cutting calories further.
- 4 Verify you're getting enough protein, fibre and sleep — plateaus are often adherence problems in disguise.
- 5 Consider a planned two-week break at maintenance calories; diet breaks can restore hormones, motivation and adherence without losing progress.

Exercise for fat loss

- **Resistance training 2–4 times a week** is the priority — it decides how much of the loss is fat rather than muscle, and keeps metabolism higher.
- **Cardio** adds to the deficit and to heart health: 150–300 minutes of moderate activity weekly.
- **Steps** are the consistent background burn that survives busy weeks.
- Don't treat exercise as permission to eat more — fitness trackers routinely overestimate calories burned.

Maintenance — the part most plans skip

- 1 Transition gradually: add 100–200 kcal per week back until weight stabilises.
- 2 Keep the habits that got you here; maintenance is the same behaviour with more food.
- 3 Keep weighing weekly, and set an action threshold — if you're up 2–3 kg from your floor, tighten up for a fortnight rather than waiting.
- 4 Keep training. Muscle retention is the strongest protection against regain.
- 5 Expect fluctuation. A stable range, not a single number, is a successful outcome.

One last thing

If you have significant weight to lose, a metabolic condition, or a long history of regain, this is not a willpower problem and you don't have to solve it alone. Medical and dietetic support, including medication where appropriate, is a legitimate tool — ask for it.

Important note

This guide is general nutrition education only. It is not medical advice, a diagnosis, or a treatment plan, and it cannot account for your personal history, laboratory results, allergies or medicines. Always speak to your doctor, dietitian or pharmacist before making significant changes to your diet, supplements or activity — especially if you are pregnant or breastfeeding, have kidney, liver or heart disease, an eating disorder, or take prescription medication. If you experience severe or sudden symptoms, seek urgent medical care. Do not follow a calorie-restricted plan during pregnancy, while breastfeeding, with a history of disordered eating, or alongside medicines that require consistent intake, without professional supervision.

Questions or want a personalised plan?

This guide covers general principles. A one-to-one consultation with The Nutrition Club Clinic lets us tailor portions, meal timing, food preferences, budget and any medications or conditions to you.

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